

February 23, 2026

Thomas Keane, MD, MBA
Assistant Secretary for Technology Policy (ASTP)
Mary E. Switzer Building, Mail Stop: 7033A
330 C Street SW, Washington DC 20201

**RE: Response to Artificial Intelligence Request for Information/Document Citation
#: 90 FR 60108**

Dear Dr. Keane:

On behalf of the Behavioral Health Technology Coalition, I am writing to thank you for the opportunity to respond to the ASTP Request for Information (RFI) published on December 23, 2025. While each member organization has issued their own individual response to the RFI, this correspondence reflects the broader views of the mental health and substance use communities regarding the application of Artificial Intelligence (AI) to health care.

This correspondence contains discussions of individual AI tools along with policy recommendations regarding how some of this technology may be reimbursed by the Centers for Medicare and Medicaid Services (CMS). Noting the widespread implementation of some AI tools among some mental health and addiction treatment providers in the public and private sectors does NOT equal the endorsement of these products by ANY of the signers of this regulatory reply.

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

The biggest single barrier to the adoption AI in the delivery of mental health and addiction treatment services in the United States is that perception has completely overwhelmed reality. Specifically, the public and congressional debate about the use of consumer facing chatbots in the provision of something that no organization supporting this letter would refer to as mental health counseling has blotted out the many extremely productive AI behavioral health use cases helping patients with mental illnesses and substance use disorders in America every hour of every day.

In turn, the biggest barrier is feeding our biggest fear: a replay of the 2009 HITECH Act scenario whereby the CMS and Congress omit behavioral health providers as they undertake the process of defining reimbursement mechanisms either for individual use cases or the AI field as whole – in this instance because of a single minded media focus on tragic examples (e.g., suicides, psychotic episodes) resulting from the use of chatbots.

601 Massachusetts Ave, NW, Suite 520W, Washington, DC 20001 | 202.331.1120 | www.bhitcoalition.org

American Foundation for Suicide Prevention • American Psychological Association • Association for Behavioral Health and Wellness
• Centerstone • Jewish Federations of North America • Meadows Mental Health Policy Institute • Mental Health America • National
Alliance on Mental Illness • National Association of Counties • National Association of County Behavioral Health & Developmental
Disability Directors • National Association for Behavioral Healthcare • National Association of State Alcohol/Drug Abuse Directors •
National Association of Social Workers • National Council for Mental Wellbeing • Netsmart

Chatbots: While there is no unified position within the Coalition regarding how chatbots should be overseen at the state or federal levels, Dr. Vaile Wright’s testimony on behalf of the American Psychological Association before the House Energy and Commerce Health Subcommittee on September 3, 2025, offers an example of a potential path forward. Dr. Wright recommended the following actions:

- **“Establish Clear Regulatory Guardrails.** We need a robust federal framework that ensures safety and efficacy, including prohibiting the misrepresentation of AI as licensed professionals and mandating transparency and human oversight over clinical decisions.
- **Protect Vulnerable Populations, Especially Youth.** Adolescents are in a critical developmental period. We must require age-appropriate safeguards and robust data protections to support their healthy development.
- **Prioritize Equity and Mitigate Harm.** We must require that AI models undergo rigorous, independent testing for harms across diverse populations before they are deployed.
- **Invest in Research and AI Literacy.** The pace of AI development is outpacing research. We need a significant federal investment in independent research to understand AI’s impacts, paired with comprehensive AI literacy education.”

Dr. Tom Insel, the former Director of the National Institute of Mental Health (NIMH), in a Substack article entitled, “Chatbot Safety: Red Teaming, Jailbreaking and Real World Use” (Jan 26, 2026) also offered another important insight. Insel cited estimates that a stunning 28 million Americans are using just one chatbot for emotional or psychological help. “A [RAND study](#) reported that 13.1% of young people (22.2% for ages 18-21) used a chatbot for mental health advice and 92.7% found the advice somewhat or very helpful. It’s a tough call, but my guess is that currently there are still fewer people seeking mental health care from chatbots than from human therapists. But not by much. And not for long.”

Dr. Insel distinguishes between general use consumer-facing products like ChatGPT and bespoke models like Ash, Therabot, Flourish and Slingshot that are built for one thing: psychological support. It is up to the Food & Drug Administration (FDA) or other regulatory bodies to determine the safety and efficacy of these products. But it’s instructive how Dr. Insel concluded his Substack piece: “The future of mental health may not be human *or* machine. It may be human *and* machine, thoughtfully aligned. The question is not whether people will keep turning to chatbots. They already prefer chatbots as non-judgmental, ever-present, trusted supports. The question is whether we will build them wisely enough to deserve that trust.”

AI Behavioral Health Use Cases: The attached summary developed by Leavitt Partners in 2024 outlines a wide array of AI applications in the mental health and substance use

space. **AI AND BEHAVIORAL HEALTH IS NOT EXCLUSIVELY ABOUT CHATBOTS.** Among the most widely deployed currently are:

■ **AI Scribe/Ambient Listening/Clinical Documentation :** The behavioral health field is experiencing a generational workforce crisis. AI tools can now automatically generate clinical documentation in recognized notes formats during or immediately following patient sessions. The tool summarizes medical histories including symptoms, screening results, medication interactions and side effects, and dosage modification recommendations supporting compliance with health record requirements greatly relieving provider workload.

■ **Intake Management:** AI tools can automate the many clinical intake processes replacing manual data entry, collecting standardized patient information support clinical decision making by freeing clinical staff from administrative duties and ensuring consistent capture of accurate intake data.

Similar AI tools – also applied at intake – could assist with the integration of behavioral health and primary care. Line primary care physicians and nurse practitioners could employ AI tools to automate screening for depression, anxiety and suicidality. Collaborative Care teams could employ AI dashboards for coordination and mood disorder tracking.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

The impact of AI on behavioral health is no different than its projected impact on the broader U.S. economy. These technologies could help a beleaguered workforce, potentially produce improved clinical outcomes and generate unheard of efficiencies. But these desired end states require new thinking on the part of CMS. For illustration purposes, at present, most providers pay directly out of their overhead margins to purchase AI scribe tools.

However, AI requires a reexamination of how CMS approaches Medicare payment policy. Strictly speaking, ambient listening are tools are not just “note taking devices” Because of the rich clinical information they contain, these AI tools are essential to – and indistinguishable from – the Evaluation & Management Process engaged in by mental health and addiction treatment clinicians every day.

Therefore, in response to RFI Question # 2, the BHIT Coalition specifically recommends that Evaluation and Management (E/M) codes (99202–99499) be expanded to authorize the reimbursement of AI Scribe/Ambient Listening/Clinical Documentation Tools.

601 Massachusetts Ave, NW, Suite 520W, Washington, DC 20001 | 202.331.1120 | www.bhitcoalition.org

American Foundation for Suicide Prevention • American Psychological Association • Association for Behavioral Health and Wellness • Centerstone • Jewish Federations of North America • Meadows Mental Health Policy Institute • Mental Health America • National Alliance on Mental Illness • National Association of Counties • National Association of County Behavioral Health & Developmental Disability Directors • National Association for Behavioral Healthcare • National Association of State Alcohol/Drug Abuse Directors • National Association of Social Workers • National Council for Mental Wellbeing • Netsmart

4. For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, and/or prize competitions)

The Coalition has a twofold answer to this question – a broad policy response and an immediate practical reply. First, from a policy perspective, HHS should consider the establishment of a formal framework to define, evaluate and validate “mental health efficacy” for generate AI-enabled tools by integrating clinical outcomes metrics, user safety benchmarks, and AI-specific performance standards. The framework would present an opportunity to drive alignment across agencies (e.g., CMS, ASTP/ONC, SAMHSA) on adverse events, longitudinal endpoints and real-world data collection.

But there are more immediate opportunities. The Center for Medicare and Medicaid Innovations (CMMI) has an Integration Behavioral Health (IBH) Demonstration Notice of Funding Opportunity (NOFO) currently open for solicitation. CMMI should be encouraged to make the AI behavioral health use cases outlined the two-page attachment eligible for immediate reimbursement under the IBH model. Further, CMS should consider making behavioral health providers of all types eligible for contracts, grants or cooperative agreements to field – on a wide scale – the AI tools described in the Leavitt Partners summary. If this technology really is capable of generating efficiencies, CMS shouldn’t be afraid to test it with some of the most challenging patient populations.

5. How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

Again, the Coalition has a twofold response to this question. HHS/CMS/ASTP should consider the creation of a unified, federally supported registry to monitor the real-world safety and efficacy of generative AI tools used in mental health and substance use services. This would require collaboration with developers, EHR vendors, and digital therapeutic companies.

But, as with Question # 4, there are practical considerations. First, ASTP and SAMHSA just launched the USDCI+ BHIT Pilot in nine (9) sites. CMS and ASTP will certainly want to wait for the results of that pilot project before moving forward to any AI clinical certification and standards related activities. Second, our assumption is that the federal agencies involved will collaborate with the major private sector certification bodies including CARF, NCQA, etc.

6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools

601 Massachusetts Ave, NW, Suite 520W, Washington, DC 20001 | 202.331.1120 | www.bhitcoalition.org

American Foundation for Suicide Prevention • American Psychological Association • Association for Behavioral Health and Wellness • Centerstone • Jewish Federations of North America • Meadows Mental Health Policy Institute • Mental Health America • National Alliance on Mental Illness • National Association of Counties • National Association of County Behavioral Health & Developmental Disability Directors • National Association for Behavioral Healthcare • National Association of State Alcohol/Drug Abuse Directors • National Association of Social Workers • National Council for Mental Wellbeing • Netsmart

would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

The Coalition cannot express an opinion on which AI tools met or exceeded performance and cost expectations. As noted in the response to Question # 1, some of the most widely utilized AI technology in the behavioral health space are AI scribes and intake management tools.

With respect to novel AI tools that have the greatest potential to improve health care outcomes and help reduce costs, perhaps the most interesting (without the intention of providing an endorsement of any individual product) are AI technologies that identify and aid clinical diagnosis of mental illnesses and developmental disorders (e.g., autism) more accurately efficiently and accurately. For example, Tardive Dyskinesia (TD) is a highly disabling movement disorder that is a common side effect of anti-psychotic medications widely prescribed to treat schizophrenia and bipolar disorder. It requires at least 20 minutes for a psychiatrist to administer the Abnormal Involuntary Movement Scale (AIMS) test in order to diagnose TD. However, there are now video AI tools that screen for the presence of TD via facial recognition clues in minutes.

7. Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

In the context of behavioral health organizations, this question gets it wrong. Its not “administrative” hurdles that obstruct the adoption of AI in clinical care. It is lack of clear reimbursement pathways. Period. Full Stop.

8. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

Finally, this question calls for subjective responses and its difficult for a Coalition that includes multiple patient advocacy organizations to respond comprehensively. In general, people with mental health and substance use disorders hope that AI tools, at a minimum, can provide this population with greater control over their own health care records and perhaps better coordination across the healthcare system.

The concerns of the behavioral health advocacy organization, again as a general proposition, revolve around that lack of input that people with lived experience have had in the development of the technology providing the basis for AI health care tools. In addition, privacy concerns are pervasive. Once mental health and substance use patient data is shared with an AI tool, who controls it?

- Who is the data shared with?
- Does law enforcement have access to it?
- Can it be used in employment decisions or family court?

Again, thank you for the opportunity to respond to this important Request for Information. We end this correspondence where it began. **The Behavioral Health Information Technology Coalition strongly urges both ASTP and CMS not to repeat the policy mistakes of the past.**

Sincerely,

Alfonso V Guida, Jr.
Behavioral Health IT Coalition Coordinator
Email: aguida@guidelobby.com
Phone: 202.253.3386